

# MediLink

## Legacy Championship Handbook

Elite Chapter Delegation Events • Detailed Rules, Simulations, Rubrics, and Championship Design

Five Legacy events • In-person • On-site advanced release • Chapter-owned elite circuit

**Purpose.** Legacy events are not “harder normal events.” They are selective, chapter-owned championship simulations that demand synthesis across medicine, finance, leadership, technology, policy, and communication. A Legacy team must make decisions under ambiguity, defend tradeoffs, absorb new facts, and maintain a coherent strategy under pressure.

**Event identity.** Each Legacy competition is built as a recognizable flagship experience with a permanent brand, an annual hyper-specific fact pattern, multi-layer decision-making, a live challenge mechanism, and a rubric that rewards substantive reasoning over presentation polish.

# Legacy Competition System

Legacy events exist to create chapter identity, retention, mentorship, prestige, and elite collaborative challenge. They are designed for a small selected delegation and must remain fundamentally team-based, multi-round, and difficult.

## Delegation and Entry Rules

- Each school chapter selects a maximum of eight Legacy-eligible students for the season, organized as two groups of up to four students.
- For any one Legacy event, a chapter may enter only one of its two groups. The other group may not enter the same Legacy event.
- The selected eight form the chapter's season-long Legacy delegation. Except for narrowly documented, approved circumstances, substitutes may not be added after the season begins.
- Legacy delegates may also compete in normal events. Legacy performance primarily represents the chapter, while individual excellence may be recognized separately or considered for the biennial invitational.
- All Legacy rounds occur in person. Each event uses the On-site: Advanced Release model: the broad domain is announced earlier; the full confidential packet releases two hours before the judged simulation.

## Championship Advancement and Cumulative Points

Legacy events use a selective cumulative championship ladder. Regionals eliminate all but the top five entries; States determine the top three cumulative standings that advance to Nationals; Nationals determines the champion by cumulative standing. There is no consolation bracket.

| Placement | Regional points | State points | National points |
|-----------|-----------------|--------------|-----------------|
| 1st       | 12              | 20           | 35              |
| 2nd       | 10              | 16           | 28              |
| 3rd       | 8               | 13           | 22              |
| 4th       | 6               | 10           | —               |
| 5th       | 4               | 8            | —               |

At States, all five Regional qualifiers receive State placement points, but only the top three by cumulative score advance to Nationals. At Nationals, the team with the highest cumulative score after all three rounds is the Legacy National Champion. Ties are resolved by the later-round score, then technical-content score, then oral-defense score, then a designated tie-break challenge.

## Common Legacy Integrity Rules

- During the two-hour supervised preparation period, personal devices, outside communication, internet research, generative AI, and outside coaching are prohibited unless specifically authorized by the annual event guide.
- Teams may use only the supplied packet, allowed tools, official reference materials, and materials created during supervised preparation.
- Each team must submit its preparation notes and all required written materials before entering the judged phase.
- Judges may question any delegate about any part of the team's reasoning. A team cannot conceal nonparticipation behind a polished presentation.
- Each event includes case facts that may be incomplete or conflicting. Teams must distinguish facts, assumptions, and recommendations.

## Legacy Event Identity Map

| Event            | Primary domain                       | Team role           | Defining live mechanic                                  |
|------------------|--------------------------------------|---------------------|---------------------------------------------------------|
| The Atlas Docket | AI, clinical safety, law, governance | Hospital AI counsel | The Bench: judicial questioning and evidence objections |

| Event                | Primary domain                                   | Team role                       | Defining live mechanic                                         |
|----------------------|--------------------------------------------------|---------------------------------|----------------------------------------------------------------|
| The Covenant Table   | Capital finance, access, health-system strategy  | Capital allocation council      | The Red Ledger: credit/community shock and forced reallocation |
| Black Box Protocol   | Clinical technology, cybersecurity, procurement  | Technology review board         | Kill Switch: deployment incident and governance drill          |
| The Last Mile Accord | Reimbursement, contracting, population health    | Payment-design delegation       | The Corridor: negotiation, arbitration, and contract shock     |
| Nightfall Command    | Incident command, operations, workforce, finance | Hospital command executive team | The Nightfall Clock: escalating crisis injects                 |

# L1. The Atlas Docket

HEALTH LAW, CLINICAL AI, EQUITY, FINANCE, AND REGULATORY GOVERNANCE | MOCK APPELLATE REGULATORY HEARING  
| TEAM-ONLY: 2–4 LEGACY DELEGATES

**Team role:** Hospital AI governance counsel    **Primary action:** Argue    **Live mechanic:** The Bench: judicial questioning plus evidence objection round

A fictional regional health system has deployed “Atlas,” an AI-supported emergency-department acuity and routing platform. Atlas shortens waiting times, reduces avoidable admissions, and improves throughput, but an independent audit suggests it may under-prioritize patients from particular demographic groups and may have been trained on incomplete local data. After a serious alleged harm, a coalition challenges the system’s continued deployment. Teams serve as counsel before a specialized appellate-regulatory panel deciding whether Atlas may remain in use, under what conditions, and with what financial, clinical, and legal safeguards.

## Why This Event Is Legacy-Level

This event requires teams to connect technical evidence with real organizational consequences. A winning solution must be defensible from multiple viewpoints, withstand new information, and show why its strategy remains safer, fairer, or more viable than competing approaches. The event intentionally has no single obvious solution.

## Core Decision Layers

- The clinical question: does the tool improve actual patient care, or does its observed efficiency conceal unacceptable safety risk?
- The data question: what do sensitivity, calibration, subgroup performance, missing data, local validation, and workflow behavior actually establish?
- The governance question: who is accountable when clinicians, administrators, vendors, and algorithms share decision influence?
- The financial question: how should a hospital weigh savings, licensing costs, liability exposure, staffing pressure, and the cost of alternative safeguards?
- The remedy question: approve, restrict, redesign, suspend, independently audit, pilot, or terminate—and why?

## Preparation Before Competition

- Study AI and clinical-decision-support concepts; evidence interpretation; algorithmic bias; privacy and data governance; hospital operations; liability and regulation; and persuasive hearing practice. The broad annual domain is published early. The full record, exhibits, witness summaries, contract excerpts, audit tables, and procedural question release two hours before the hearing in supervised preparation.

## Competition Process

- Two-hour supervised case review and role allocation.
- Teams submit a written appellate brief and exhibit index before entering the hearing room.
- Hearing sequence: opening argument; panel questioning; evidence/admissibility objection round; opposing-case rebuttal or respondent defense; remedy argument; closing.
- During the hearing, the Bench may disclose a new audit exhibit, vendor contract clause, patient testimony, or sentinel-event update.

## Required Deliverables

- Written appellate/regulatory brief.
- Exhibit and evidence theory map.
- Clinical-safety and equity analysis.

- Governance/remedy framework.
- Oral hearing performance and individual cross-questioning.

## Championship Rubric — 100 Points

| Criterion                                        | Points | What elite performance demonstrates                                                                                                                                                                                          |
|--------------------------------------------------|--------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Legal-regulatory theory and remedy design        | 25     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Clinical validity, safety, and evidence analysis | 20     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Equity, data governance, and accountability      | 20     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Financial and operational consequences           | 15     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Advocacy, rebuttal, and response to the Bench    | 15     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Individual mastery and team coordination         | 5      | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |

## Judge Standard and Annual Expansion

**Judge standard.** Strong teams do not simply offer more ideas. They make disciplined choices, name what they will not do, disclose assumptions, protect against foreseeable harm, and present an implementation pathway that a real organization could follow. **Annual expansion.** The event remains reusable because the annual technology, health system, claimed harm, and regulatory posture can change while the “Atlas Docket” identity remains.

## Event Rules and Design Boundaries

- This is a fictional educational hearing, not legal advice.
- Teams are scored on the strength and coherence of their analysis, not on personal views about AI.
- A strong team may argue for conditional use, suspension, redesign, or termination if the record supports it.

## Round Difficulty Escalation

| Round     | Expected event design                                                                                                                                           |
|-----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Regionals | Core fact pattern; teams must demonstrate sound framework, technical competence, and an initial defensible strategy.                                            |
| States    | More complex data, stronger stakeholder conflict, tighter resource constraints, and a consequential live update.                                                |
| Nationals | Full multi-layer simulation with advanced evidence, high-stakes tradeoffs, intense questioning, and a final change that tests whether the strategy truly holds. |

## L2. The Covenant Table

MEDICINE, CAPITAL FINANCE, HOSPITAL STRATEGY, ACCESS, AND EXECUTIVE LEADERSHIP | FIVE-YEAR CAPITAL-ALLOCATION SUMMIT | TEAM-ONLY: 2–4 LEGACY DELEGATES

**Team role:** Health-system capital allocation council    **Primary action:** Allocate    **Live mechanic:** The Red Ledger: fixed capital tokens plus a late credit shock

Teams become the Capital Allocation Council for a fictional health system facing aging infrastructure, tight debt capacity, uneven access, workforce gaps, and competing clinical investments. They must decide what the system funds, phases, partners for, redesigns, defers, protects, or closes over five years. The challenge is not to fund everything; it is to make painful choices without treating patients, communities, or staff as spreadsheet entries.

### Why This Event Is Legacy-Level

This event requires teams to connect technical evidence with real organizational consequences. A winning solution must be defensible from multiple viewpoints, withstand new information, and show why its strategy remains safer, fairer, or more viable than competing approaches. The event intentionally has no single obvious solution.

### Core Decision Layers

- Teams review service-line margin, utilization, quality, workforce, payer, access, debt, and community data.
- They construct a five-year portfolio within a hard capital ceiling and financing limit.
- They choose among competing opportunities such as behavioral-health capacity, rural emergency stabilization, maternal transfer networks, imaging replacement, oncology expansion, cybersecurity, home-based chronic care, primary-care access, debt reduction, or strategic partnerships.
- They must explain clinical benefit, equity, opportunity cost, financing structure, risk, and first-year execution.

### Preparation Before Competition

- Study healthcare finance, capital budgeting, debt and cash concepts, hospital operations, reimbursement, community access, quality measures, and executive communication. The annual health-system type and broad capital challenge are announced in advance. The full financial, clinical, and community packet releases two hours before the summit.

### Competition Process

- Two-hour supervised portfolio build.
- Teams place finite capital tokens across investment categories and submit a balanced five-year allocation table.
- Board summit: presentation to clinical, finance, labor, patient, and community stakeholders.
- Red Ledger shock: a bond downgrade, payer shift, cyberattack, physician exit, major employer closure, or emergency-capacity failure changes borrowing cost or community need.
- Teams must rebalance their portfolio publicly and defend what they are willing to defer.

### Required Deliverables

- Five-year capital portfolio.
- Financing plan: cash, debt, lease, partnership, grant, philanthropy, or staged investment.
- Clinical-access and equity impact statement.
- Risk register and first-90-days execution plan.
- Community/employee board briefing.

### Championship Rubric — 100 Points

| Criterion                                        | Points | What elite performance demonstrates                                                                                                                                                                                          |
|--------------------------------------------------|--------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Capital-allocation logic and financial viability | 30     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Clinical benefit, safety, access, and equity     | 25     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Use of data and explicit assumptions             | 15     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Financing strategy and risk management           | 15     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Response to the Red Ledger shock                 | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Board communication and team leadership          | 5      | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |

## Judge Standard and Annual Expansion

**Judge standard.** Strong teams do not simply offer more ideas. They make disciplined choices, name what they will not do, disclose assumptions, protect against foreseeable harm, and present an implementation pathway that a real organization could follow. **Annual expansion.** The annual system can change—rural nonprofit network, pediatric system, safety-net organization, urban academic center, merger candidate—without weakening the event identity.

## Event Rules and Design Boundaries

- No portfolio is pre-designated as “correct.” Judges score coherent tradeoffs, data discipline, and viable implementation.
- Teams cannot earn top marks by maximizing margin while ignoring essential access, nor by preserving every service without a funding explanation.

## Round Difficulty Escalation

| Round     | Expected event design                                                                                                                                           |
|-----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Regionals | Core fact pattern; teams must demonstrate sound framework, technical competence, and an initial defensible strategy.                                            |
| States    | More complex data, stronger stakeholder conflict, tighter resource constraints, and a consequential live update.                                                |
| Nationals | Full multi-layer simulation with advanced evidence, high-stakes tradeoffs, intense questioning, and a final change that tests whether the strategy truly holds. |

# L3. Black Box Protocol

TECHNOLOGY, MEDICINE, CYBERSECURITY, CLINICAL EVIDENCE, PROCUREMENT, AND ETHICS | CLINICAL TECHNOLOGY DEPLOYMENT AND FAILURE SIMULATION | TEAM-ONLY: 2–4 LEGACY DELEGATES

**Team role:** Independent clinical technology review board    **Primary action:** Validate    **Live mechanic:** Kill Switch: live incident and deployment-governance drill

A hospital system is considering a high-stakes technology—such as sepsis prediction, radiology prioritization, maternal-risk modeling, remote monitoring, medication-error detection, AI documentation, or prior-authorization automation. The product promises savings, speed, and improved detection, but the evidence may be incomplete, biased, non-generalizable, insecure, or incompatible with the real workflow. Teams must decide whether and how the system should deploy it.

## Why This Event Is Legacy-Level

This event requires teams to connect technical evidence with real organizational consequences. A winning solution must be defensible from multiple viewpoints, withstand new information, and show why its strategy remains safer, fairer, or more viable than competing approaches. The event intentionally has no single obvious solution.

## Core Decision Layers

- Technical performance must be separated from clinical usefulness.
- Teams audit validation results, subgroup performance, calibration, data provenance, missingness, workflow fit, privacy, cybersecurity, vendor terms, and financial impact.
- They define a deployment scope, human-oversight model, success thresholds, monitoring plan, audit rights, incident protocol, and shutdown authority.
- They may approve, reject, pilot, restrict, redesign, defer, or replace the technology with a nontechnical alternative.

## Preparation Before Competition

- Study clinical AI and digital-health evaluation, basic diagnostic-performance concepts, bias, privacy, cybersecurity, interoperability, workflow design, procurement, contracts, and health-system finance. The annual technology class is announced in advance; the full dossier releases two hours before competition.

## Competition Process

- Two-hour supervised technical and clinical review.
- Teams submit a “go/no-go/conditional-go” decision memo and deployment map.
- The review board hearing tests contract terms, oversight, workflow impacts, and financial assumptions.
- Kill Switch drill: the system experiences a false negative, subgroup disparity, cyber incident, vendor acquisition, dashboard outage, clinician work-around, or data-sharing breach.
- Teams must decide whether to pause, modify, disclose, audit, continue, or terminate deployment—and communicate that decision to patients, clinicians, executives, and the vendor.

## Required Deliverables

- Formal recommendation and rationale.
- Deployment map and human-in-the-loop workflow.
- Validation and equity audit plan.
- Contract redline or procurement memorandum.
- Financial sensitivity analysis.
- Incident response and public-facing statement.

## Championship Rubric — 100 Points

| Criterion                                          | Points | What elite performance demonstrates                                                                                                                                                                                          |
|----------------------------------------------------|--------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Clinical validity and patient safety               | 25     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Technical/data-quality analysis                    | 20     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Equity, privacy, cybersecurity, and governance     | 20     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Deployment, workflow, and accountability design    | 15     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Financial, contractual, and procurement reasoning  | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Kill Switch response and stakeholder communication | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |

## Judge Standard and Annual Expansion

**Judge standard.** Strong teams do not simply offer more ideas. They make disciplined choices, name what they will not do, disclose assumptions, protect against foreseeable harm, and present an implementation pathway that a real organization could follow. **Annual expansion.** The annual fact pattern can change completely while retaining the Black Box Protocol structure.

## Event Rules and Design Boundaries

- This event is not a generic “AI debate.” Teams must make an operational decision and define exact deployment conditions.
- A recommendation to reject technology can score at the highest level if the case evidence supports it.
- Judges reward teams that recognize when workforce, process redesign, or simpler technology is better than automation.

## Round Difficulty Escalation

| Round     | Expected event design                                                                                                                                           |
|-----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Regionals | Core fact pattern; teams must demonstrate sound framework, technical competence, and an initial defensible strategy.                                            |
| States    | More complex data, stronger stakeholder conflict, tighter resource constraints, and a consequential live update.                                                |
| Nationals | Full multi-layer simulation with advanced evidence, high-stakes tradeoffs, intense questioning, and a final change that tests whether the strategy truly holds. |

# L4. The Last Mile Accord

POPULATION HEALTH, PAYER-PROVIDER CONTRACTING, CHRONIC CARE, REIMBURSEMENT, NEGOTIATION, AND JUSTICE |  
VALUE-BASED-CARE ARBITRATION AND CONTRACT NEGOTIATION | TEAM-ONLY: 2–4 LEGACY DELEGATES

**Team role:** Multi-stakeholder payment-design delegation    **Primary action:** Negotiate    **Live mechanic:** The Corridor:  
closed-door negotiation followed by public arbitration

A payer, a physician network, a safety-net hospital, and a patient coalition are trapped in a dispute over a value-based chronic-disease contract. The existing agreement rewards some outcomes but may penalize providers who serve complex patients, underfund social needs, increase administrative burden, or create incentives to avoid high-risk people. Teams must design—or arbitrate—a durable payment accord that aligns quality, access, risk, accountability, and financial sustainability.

## Why This Event Is Legacy-Level

This event requires teams to connect technical evidence with real organizational consequences. A winning solution must be defensible from multiple viewpoints, withstand new information, and show why its strategy remains safer, fairer, or more viable than competing approaches. The event intentionally has no single obvious solution.

## Core Decision Layers

- Teams interpret quality metrics, risk adjustment, utilization, patient complexity, shared savings, penalties, care-management costs, pharmacy costs, access protections, and administrative burden.
- They decide which outcomes count, who bears downside risk, how patient complexity is adjusted, how disputes are appealed, and how savings or losses are distributed.
- They must protect against gaming, under-service, cherry-picking, and metric fixation while preserving a financially credible contract.

## Preparation Before Competition

- Study health insurance and reimbursement, value-based care, risk adjustment, quality measurement, provider incentives, negotiation, chronic-disease management, health equity, and contract design. The broad clinical population and payment domain are announced early. The full data packet and stakeholder mandates release two hours before competition.

## Competition Process

- Two-hour supervised analysis and strategy build.
- Part I: closed-door negotiation with judges acting as stakeholder representatives; teams may make offers, concessions, and conditional proposals.
- Part II: public arbitration hearing where teams defend the proposed contract to an independent panel and patient advocates.
- The Corridor update introduces an adverse-selection pattern, a quality-metric failure, a new expensive therapy, or a budget shock that requires contract revision.

## Required Deliverables

- Term sheet for the payment accord.
- Quality and risk-adjustment framework.
- Patient-access/equity protections.
- Shared-savings, penalty, and appeals structure.
- Negotiation log and public arbitration defense.

## Championship Rubric — 100 Points

| Criterion                                        | Points | What elite performance demonstrates                                                                                                                                                                                          |
|--------------------------------------------------|--------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Payment design and financial logic               | 25     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Clinical quality, access, and equity protections | 25     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Risk adjustment and incentive analysis           | 20     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Negotiation and stakeholder management           | 15     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Contract feasibility and governance              | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Response to Corridor update                      | 5      | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |

## Judge Standard and Annual Expansion

**Judge standard.** Strong teams do not simply offer more ideas. They make disciplined choices, name what they will not do, disclose assumptions, protect against foreseeable harm, and present an implementation pathway that a real organization could follow. **Annual expansion.** Annual versions may focus on diabetes, heart failure, maternal care, behavioral health, oncology navigation, or another chronic/population-health context.

## Event Rules and Design Boundaries

- This event is neither a standard debate nor a generic insurance case: teams must produce negotiated contract language and live concessions.
- There may be several valid contracts; judges score consistency, protections against perverse incentives, and financial realism.

## Round Difficulty Escalation

| Round     | Expected event design                                                                                                                                           |
|-----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Regionals | Core fact pattern; teams must demonstrate sound framework, technical competence, and an initial defensible strategy.                                            |
| States    | More complex data, stronger stakeholder conflict, tighter resource constraints, and a consequential live update.                                                |
| Nationals | Full multi-layer simulation with advanced evidence, high-stakes tradeoffs, intense questioning, and a final change that tests whether the strategy truly holds. |

# L5. Nightfall Command

CLINICAL OPERATIONS, INCIDENT COMMAND, HOSPITAL FINANCE, WORKFORCE, LOGISTICS, AND PUBLIC TRUST |  
FOUR-ROUND HOSPITAL CRISIS COMMAND SIMULATION | TEAM-ONLY: 2-4 LEGACY DELEGATES

**Team role:** Hospital incident-command executive team    **Primary action:** Command    **Live mechanic:** The Nightfall Clock: escalating real-time crisis injects

A hospital must maintain safe care while multiple failures converge. Teams serve as the incident-command executive group for a fictional health system confronting a crisis such as ransomware plus mass casualties, oxygen disruption during a winter storm, neonatal infection concern, supply contamination, behavioral-health surge, medication-system outage, or respiratory demand spike. The event rewards disciplined command, not flashy improvisation.

## Why This Event Is Legacy-Level

This event requires teams to connect technical evidence with real organizational consequences. A winning solution must be defensible from multiple viewpoints, withstand new information, and show why its strategy remains safer, fairer, or more viable than competing approaches. The event intentionally has no single obvious solution.

## Core Decision Layers

- Teams establish authority, assign functional roles, identify immediate safety threats, allocate beds/staff/supplies/cash, set service priorities, protect vulnerable patients, communicate internally and externally, and plan recovery.
- Each choice creates consequences: stopping elective care may protect capacity but lose revenue; using agency staff may preserve coverage but raise cost and handoff risk; public statements may protect trust or create exposure.
- The team must make decisions on a time horizon of 30 minutes, 24 hours, 72 hours, and post-incident recovery.

## Preparation Before Competition

- Study incident command, hospital operations, clinical continuity, emergency logistics, staffing, bed management, communications, finance, risk, quality improvement, and ethical resource allocation. The broad crisis domain is announced in advance. The full incident packet releases two hours before the simulation.

## Competition Process

- Two-hour supervised initial planning.
- Round 1 — Stabilize: establish command structure and immediate patient-safety actions.
- Round 2 — Escalate: a new clinical, technical, staffing, or supply failure appears.
- Round 3 — Scrutiny: media, regulator, board, labor, or patient advocate challenges the team.
- Round 4 — Recover: teams explain restoration, financial mitigation, accountability, and system redesign. The Nightfall Clock limits response windows and forces clear prioritization.

## Required Deliverables

- 30-minute stabilization plan.
- 24-hour and 72-hour operational plans.
- Incident-command chart and resource-allocation table.
- Service-prioritization matrix: continue, modify, defer, transfer, or suspend.
- Staff/patient/public communications.
- Board/CFO financial brief and after-action review.

## Championship Rubric — 100 Points

| Criterion                                        | Points | What elite performance demonstrates                                                                                                                                                                                          |
|--------------------------------------------------|--------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Immediate patient safety and continuity of care  | 25     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Incident-command structure and leadership        | 20     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Resource allocation and operational feasibility  | 15     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Workforce strategy and staff safety              | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Financial resilience and decision transparency   | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Communication, equity, and public trust          | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |
| Response to escalating injects and recovery plan | 10     | Uses the case record precisely; makes explicit tradeoffs; integrates medicine, finance, ethics, and operations where relevant; adapts decisively to new facts; and withstands direct questioning from multiple stakeholders. |

## Judge Standard and Annual Expansion

**Judge standard.** Strong teams do not simply offer more ideas. They make disciplined choices, name what they will not do, disclose assumptions, protect against foreseeable harm, and present an implementation pathway that a real organization could follow. **Annual expansion.** The annual scenario may change radically while retaining the Nightfall Command structure.

## Event Rules and Design Boundaries

- This is deliberately the most interdependent Legacy event: no single student can credibly carry clinical, operations, finance, and communication responsibilities alone.
- Judges should score explicit decision logic and responsible adaptation, not whether teams predict every later inject.

## Round Difficulty Escalation

| Round     | Expected event design                                                                                                                                           |
|-----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Regionals | Core fact pattern; teams must demonstrate sound framework, technical competence, and an initial defensible strategy.                                            |
| States    | More complex data, stronger stakeholder conflict, tighter resource constraints, and a consequential live update.                                                |
| Nationals | Full multi-layer simulation with advanced evidence, high-stakes tradeoffs, intense questioning, and a final change that tests whether the strategy truly holds. |